

SECOND AMENDED STATEMENT OF FACTS AND CONTENTIONS (FORM 9A)

WC/2024/227 - Cory Lea Shepherd (Appellant) v Workers' Compensation Regulator (Respondent). Section 549, Workers' Compensation and Rehabilitation Act 2003. The numbers in square brackets, for example [¶ 40], are the paragraph numbers of the Appellant's notice to admit facts served on the Respondent on 28 August 2026. The Respondent responded to that notice on 8 September 2026.

PART A

Appellant: Cory Lea Shepherd. Employer: Metro South Hospital and Health Service, Logan Hospital Switchboard. Role: Administration Officer, Switchboard Services, classification AO3, a continuous shift working role. [¶¶ 1 to 4, 13] The Appellant was a worker within the meaning of section 11 of the Act. Three movement forms approved in 2026 record the shift arrangement as "Continuous Shift Worker", and the Respondent does not allege that the Appellant ceased to be a continuous shift worker at any time before 18 June 2024. [¶¶ 14 to 16] Injury: major depressive disorder with anxious distress (DSM-5 296.23). The treating psychiatrist recorded the diagnosis in writing on 24 October 2024 and gave it the code DSM-5 296.23 in his report of 13 February 2025. The injury is a diagnosed medical condition. It arose from the Appellant's employment over the course of conduct pleaded at Part B.2 below, between about June 2023 and June 2024. It is not pleaded as arising from an event on a single day. The date of injury is 18 June 2024, as recorded on the work capacity certificates. The injury arose out of, or in the course of, that employment, and the employment was a significant contributing factor to it: section 32(1) of the Act.

PART B - MATERIAL FACTS

Each fact set out below that carries a paragraph number in square brackets was admitted by the Respondent, to the extent recorded in its response of 8 September 2026. A statement without a bracket is drawn from a document identified in the Schedule of Documents enclosed with the Appellant's letter of 1 October 2026 (the Schedule) or is the Appellant's own evidence; the register at Tab 31, the call statistics and the account of travel and sleep in Stressor 3 are of that kind, and facts 228 to 231 were not admitted. The Commission is asked to decide from the admitted facts what the circumstances were, whether they were reasonable, and how much they contributed to the injury.

How this Part is arranged. Each stressor is pleaded so that it can be decided on its own facts. Where one admitted fact matters to more than one stressor, it is repeated on purpose. The repetition does not allege separate injuries and does not count causation twice. It shows the different part the same event plays in each stressor within the whole course of conduct. The medical material is placed between the facts only to show what was reported at the time, the diagnosis, capacity and treatment history. It is not used to turn a clinical history into proof of an event that is otherwise disputed. The numbered themes organise the case; the lettered particulars identify dated events or connected event sequences. Unlettered paragraphs state duties, standards, supporting records, medical links and the pleaded response. Context-only and post-injury material retains that limited purpose.

1. The starting point - the role, and the medical record

1.1 The Switchboard receives emergency response notifications and distributes them to the response groups by category of emergency, and maintains the contact and paging information by which that is done. [¶¶ 5, 7 to 8, 10] The position is also required to work effectively with limited supervision, to meet deadlines and establish work priorities, and to work as a member of a multi-disciplinary team in a changing environment. [¶ 11]

1.2 **Prior health.** Before 18 June 2024 the Appellant had not been diagnosed with or treated for depression. The general-practice record of 16 November 2023 (Dr Nanayakkara) records poor sleep with shift work, that the Appellant could not do shifts without a good sleep, "No psychological illness such as depression/psychosis", and mood good, with melatonin and temazepam prescribed that day. [Tab M1 - general-practice records, Our Medical Ashmore, 1 January 2023 to 1 July 2024, obtained by the Respondent under the Form 29 signed 4 July 2025; Respondent's item 11.] This record is relied on because it shows, before any claim or proceeding, that shift work was affecting the Appellant's sleep, and what was prescribed for it. What it means clinically is a matter for the treating doctors. Private entries unrelated to the injury are redacted on the extracted pages and marked as such. The records show no diagnosis of, or treatment for, a depressive

disorder before June 2024. No medication for mental health was current in May 2024 (paragraph 1.3). On the same records the Respondent's own review found that employment was a significant contributing factor to the injury (paragraph 1.8).

1.3 The referral of 16 May 2024. The referral letter of 16 May 2024 (Dr Zhao) referred the Appellant to a psychiatrist, Dr Amini, for "ongoing care and management". It lists the past medical history as the history-list items "26/10/2022 ADHD" and "26/10/2022 Anxiety". It lists the medications then current; they include no antidepressant, no anti-anxiety medication and no other medication for mental health. [Tab M1.] No appointment with the psychiatrist was obtained on that referral. The referral was made the day after the request to retract at Stressor 1(l). [¶¶ 76 to 77]

Particulars: paragraphs 76 to 77 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

1.4 Deterioration and first presentation. The Appellant last worked a shift on 3 June 2024. The date of injury recorded on the certificates is 18 June 2024. On 28 June 2024 (Dr Slawinski) the record shows "stress at work" and "upset by people not following rules", reason for visit anxiety. On 1 July 2024 (Dr Hawes) the record shows "work stress", "been there 5 years", that "they withhold pay at times, no overtime- not processed, manipulate his roster- so he works lates then earlies", and "causing anxiety". [Tab M1.] The events those records name are the matters admitted on 8 September 2026 and pleaded at Part B.2 below. The records are relied on as the record, made at the time, of what the Appellant reported before any claim decision, dismissal or proceeding. They are not relied on as a finding of how the injury was caused.

1.5 Certification. The work capacity certificates of Dr Hawes dated 1 July, 11 August and 8 September 2024, and of Dr Ki Pang dated 7 August 2024, state the date of injury as 18 June 2024, that the Appellant was first seen for this injury on 1 July 2024, and certify no functional capacity continuously from 1 July to 6 October 2024. The referral to a psychiatrist is recorded on the certificate of 8 September 2024. Review Decision 69983 records at page 17 that the certificate of 1 July 2024 indicated "there was no pre-existing factor or condition", and that "This was maintained in all later work capacity certificates". [Tab M2; Respondent's items 7 and 8.] The medication box is unticked on each certificate.

1.6 Diagnosis, 24 October 2024. At the first consultation, on the referral, the treating psychiatrist, Dr Ravikumar Bangalore Krishnaiah, told the Appellant, and confirmed in writing at 11:45 am that day, that he was "suffering from psychological injury of Major Depressive Disorder with anxiety state"; that fluoxetine was increased to two capsules "from today" and Seroquel 25 mg commenced at night, "to restore basic needs- sleep, eating and routine". The Appellant's email of 5:12 pm that day reads "I have finally been able to see a psychiatrist today". [Tab M3; Respondent's item 9.] Read with paragraph 1.5, it records the diagnosis made on 24 October 2024 of the condition first certified on 1 July 2024.

1.7 The report of 13 February 2025, its footer, and the author's position. Diagnosis: Major Depressive Disorder with anxious distress (DSM-5 296.23). The report states the severity, the effect on functioning and the treatment (fluoxetine increased to three capsules daily; quetiapine 25 mg at night). The treating clinician records the origin as "workplace stress stemming from issues with management and rostering", records that pay was "withheld or delayed for up to five months at a time" as a source of financial stress, and states that "premature exposure to the workplace is more likely result in significant deterioration". [Tab M4; Respondent's item 10.] The stressors recorded in that report match matters admitted on 8 September 2026: management and rostering, and the night-shift line; the shorter break; and pay withheld or delayed from the 5 February 2024 fortnight, uncorrected at 13 May, "claims older than 3 months" on 28 May, a claim effective 30 March recorded "Part Completed" on 30 May, and the two February claims absent from the myHR report. The report bears a footer reading "for the only reason of clinical information and not for medico-legal use". The author's emails of 5 and 8 September 2026 state of his records "You can use them according to the need to support your legal issues", and identify the report of 13 February 2025 as "the report that captures the relevant information you have requested" on the matters in issue. [Tab M5.] The report is not relied on to say which of the individual events pleaded at Part B.2 caused the injury, or how much each contributed. Nothing after 24 October 2024 is relied on as a cause of the injury.

Particulars: paragraphs 185 to 190, 193, 196 to 197, 203, 210 to 212, 220, 258 to 260 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

1.8 Records still to come, and the finding in the decision under appeal. The clinical records of the treating psychiatrist from 24 October 2024 were offered by the practice on 5 September 2026 and have been requested. They are not yet received and will be served on receipt. On the medical evidence then before it, the Respondent's review of 24 October 2024 found that the Appellant "sustained a personal injury of a

psychological nature" and stated: "Having regard to the medical evidence, I am satisfied your employment was a significant contributing factor to the psychological injury". The claim was rejected under section 32(5), not section 32(1). [Tabs M6, M8 and M9.] The hearing is de novo (a fresh hearing), so that finding does not bind the Commission. It is relied on as an admitted document.

Particulars: paragraphs 261 to 262 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

1.9 The medical evidence at hearing. No medico-legal report has been, or will be, prepared for this proceeding. The Appellant relies on the treating records and documents at Tabs M1 to M4 and M7 to M9, served with the Schedule of Medical Documents Relied Upon on 9 September 2026. He also relies on the oral evidence of the treating general practitioner, Dr Peter Hawes, and the treating psychiatrist, Dr Ravikumar Bangalore Krishnaiah. Each is named in the Appellant's list of witnesses filed 9 September 2026. Each will be asked to address, from his own records and the admitted facts put to him: the diagnosis; the history reported to him and when; whether the Appellant's employment, and in particular the matters pleaded at Stressors 1 to 3, was a significant contributing factor to the injury; and the non-employment matters recorded in the report of 13 February 2025. This paragraph gives notice of that evidence. Once hearing dates are set, the Appellant will apply for attendance notices and for leave for each practitioner to give evidence by telephone or video link at a fixed time.

2. The stressors that caused the injury (the whole course of conduct, June 2023 - June 2024)

List of stressors. WC/2024/227 - Cory Lea Shepherd v Workers' Compensation Regulator. Set out for the purposes of Part 4.8 of the Workers' Compensation Appeal Guide. There are three stressors. Each is pleaded below by dated particulars, lettered in date order; the causal particulars are identified at the head of each stressor, and the other particulars are context. An index of the particulars is at Part 3 of the Schedule enclosed with the Appellant's letter; it adds no stressor.

No.	Date or date range	Title	Description
1	18 July 2023 to 18 June 2024; context from 6 June 2023	The conditions in which the work was carried on	Database access removed and not restored; changes to process made effective the day they were notified, with no listed record of consultation; emergency calls misdirected and a directory entry not corrected; fixed office hours not stated for approximately nine months, and the written request for them met with a request to retract. Particulars 1(a) to 1(l); causal particulars 1(b), 1(d), 1(f) to 1(h), 1(k) and 1(l).
2	9 February to 5 June 2024; later correction to 2 July 2025	Remuneration	Pay wrong in four fortnights; Payroll's direction of 3 May 2024 not actioned at 13 May, still awaited on 21 May, lodged 28 May and recorded "Part Completed"; a paid leave request declined twice, with the Respondent acknowledging that the attachments were present. Particulars 2(a) to 2(o); causal particulars 2(a) to 2(i) and 2(l) to 2(n).
3	17 March to 20 May 2024; context from 7 August 2023	The roster and fatigue	A seven-hour break rostered in that role against a minimum of ten hours, or eight by written agreement, with no staff-initiated swap alleged; sick leave the next day; the fatigue enquiry answered after 23 days with a refusal; the roster decision recorded on 10 May 2024; no fatigue risk management at the Switchboard before 30 June 2024; and the Respondent's own review finding that the rostering was unreasonable management action. Particulars 3(a) to 3(h); causal particulars 3(b) to 3(h).

In summary. The injury: a psychological injury, diagnosed, with employment found by the Respondent's own review to be a significant contributing factor [¶¶ 261 to 262]. Stressor 1: a continuous 24/7 patient-safety function [¶¶ 2, 8, 289] carried on while database access was removed and not restored [¶¶ 40, 55]; process changes made effective the day they were notified, with no listed record of consultation [¶¶ 51, 181, 273]; calls misdirected and the directory not corrected [¶¶ 58, 67, 89 to 93, 109 to 110]; and the manager's fixed office hours not stated for approximately nine months [¶¶ 71 to 73]. Stressor 2: pay wrong in four fortnights [¶¶ 185 to 189]; Payroll's direction of 3 May 2024 not actioned at 13 May [¶ 193], still awaited on 21 May [¶ 195], lodged 28 May and recorded "Part Completed" [¶ 203]; and a paid leave request declined twice, with

the Respondent acknowledging that attachments were present when the later decline was made [¶¶ 116, 118, 134]. Stressor 3: a seven-hour break rostered in that role [¶ 284] against a minimum of ten hours, or eight by written agreement [¶ 285], the agreement's scope disputed by reference to the later HR email and no swap alleged [¶¶ 225, 234]; the fatigue enquiry answered after 23 days with a refusal [¶¶ 247, 249]; the employer's statement that fatigue risk management assessment was implemented after 30 June 2024, read with the May risk-matrix discussion [¶¶ 264, 269 to 271]; and the Respondent's own review finding that the rostering was unreasonable management action [¶ 260].

STRESSOR 1 - THE CONDITIONS IN WHICH THE WORK WAS CARRIED ON

Scope of Stressor 1. The causal particulars are 1(b), 1(d), 1(f) to 1(h), 1(k) and 1(l). Particulars 1(a), 1(c), 1(e), 1(i) and 1(j) preserve the contextual sequences in the existing pleading. They are not added as separate causes within this theme. Each dated particular is read with the duties and standards below.

The duty to keep the contact information accurate. A key responsibility of the position is to collate information and maintain the Omnivista database and SharePoint so that the information held within Switchboard Services is accurate and appropriate. [¶ 6]

A further key responsibility is to support hospital staff in the allocation, coordination and fault repair of all pager units, including maintaining the database and registers with accurate and current information. [¶ 7] The operational particulars below are pleaded against that duty.

How critical the function is, as the Respondent admits. On 18 February 2026 the Respondent responded to an earlier notice to admit facts in this proceeding. [¶ 283] It admitted that "Maintaining accurate contact details for medical staff is a critical function of the Switchboard to ensure effective clinical handover and patient safety". [¶ 289] In admitting that paragraph it added "and says that there was a procedure in place for this to occur", and it denied a further paragraph on the ground that there was already a procedure in place. [¶¶ 290 to 291] The procedure in place from 18 July 2023 is pleaded at 1(b). Its operation in May 2024 is pleaded at 1(h) and 1(k).

The duty to consult, and the absence of any record of consultation. Clauses 3.2.3 to 3.2.5 of the Queensland Public Health Sector Certified Agreement (No. 11) 2022 provide that the requirement of consultation is never to be treated perfunctorily or as a mere formality. They provide that consultation involves more than a mere exchange of information, and requires participants to contribute to the decision-making process "not only in appearance, but in fact". They provide that the process requires the exchange of timely information so that the parties have "an actual and genuine opportunity to influence the outcome, before a final decision is made". Clause 4.1.4 requires a business case to be tabled for consultation for significant organisational change, including major alterations to current service delivery arrangements; it is relied on only to the extent the change was of that kind. The change notified on 15 April 2024 was stated to be effective the day it was notified. [¶ 51]

The Respondent's amended List of Documents records no document recording consultation with Switchboard operators before that change. [¶ 273]

The Respondent does not allege that agreement under clause 6.2 of the Award was obtained, or that a ballot of affected employees was conducted, before it. [¶¶ 180 to 181]

The employer conducted that process on another occasion. Its Consultation outcome document defines agreement as the consent of a majority of affected employees, and records that seventeen employees were invited to vote, seventeen votes were received, and the proposal was accepted. [¶¶ 167 to 170, 176 to 177]

The safety responsibilities of the position. A key responsibility of the position is to follow defined service quality standards and occupational health and safety policies and procedures relating to the work being undertaken, to ensure high quality, safe services and workplaces. [¶ 12]

1(a) Removal of the Communication Book entry (on or around 6 June 2023). A key responsibility of the position is to maintain discretion and exercise judgement where necessary to resolve problems within the scope of the role, in situations where precedence has not been set and procedures are not defined. [¶ 9]

At 9:57 am on 6 June 2023 the Switchboard Manager wrote to Logan Switch that the book "is not used to simply put your point across or make any indirect comments towards the team". [¶¶ 148 to 149]

At 4:05 pm she wrote to the Director: "I was not aware of who put this entry in at the time but I took it out last week", and that she felt it was being used as a "burn book" more than a professional tool. [¶¶ 145 to 147]

The Respondent's pleading admits that pages were removed on or around 6 June 2023, and says the event was reported to the Director by email that day. [¶¶ 143 to 144]

On 18 February 2026 it admitted that a page was removed from the workplace communications book, and did not admit that the page contained the Appellant's handwriting "because the respondent does not have a copy of the page". [¶ 288]

The Respondent does not identify the entry removed or its author, does not allege that the removed page has been located or that any copy exists, does not allege that any entry made by the Appellant remains in

the book, and does not allege that the book ceased to be in its possession during his employment. [¶¶ 150 to 153]

The Respondent has admitted that it does not have a copy of the removed page. [¶ 288] The Respondent's amended List of Documents of 14 August 2026 does not list the Communication Book, nor any page or entry from it; that proposition (paragraph 154 of the notice) was not admitted.

1(b) Database access and the restricted correction process (18 July 2023 to 18 June 2024). On 18 July 2023 at 12:56 pm the Switchboard Telecommunications Coordinator wrote to the Appellant and the Switchboard team explaining that her role was a temporary project role focused on fixing the database. [¶ 39] She stated: "While I am fixing up the database and all of its entries, I will be removing everyone's access to the database." [¶ 40]

She stated that she had removed the Contact & Number Changes book out of the room, preferring all changes to be sent to her email. [¶ 45]

The Respondent's own review decision records the same matters. [¶¶ 46 to 48]

The Respondent does not allege that access to the database was restored to the Appellant at any time before 18 June 2024. [¶ 55] From 18 July 2023 to 18 June 2024 is eleven months. Nothing is alleged to have happened in that time to restore access. The duty described above remained on the position throughout that period.

The repair path, and its availability. The Coordinator's stated working hours were "every Tuesday and second Monday, 8:00 - 16:00". [¶ 41] A person wanting an entry updated was to "contact me directly, either via email or extension". [¶ 42] If an entry was needed on a day she was not present, and was urgent, it was to be sent to the Switchboard Manager. [¶ 43]

A request made "after hours (overnights, on the weekend or public holiday)" would "have to wait until either Chloe or myself are back". [¶ 44]

How long an incorrect entry stayed uncorrected depended on when those two people were available.

1(c) Roster concerns and the move to full time (7 August to 16 October 2023). On 7 August 2023 at 12:21 pm the Appellant wrote to the Manager, the Director, Ms Conaghan and Ms Smith, marked of High importance, asking to "formally adhoc an additional 2 shifts per fortnight" under clause 11.7 of the agreement, and: "Please give me some space and stop with any further communication as I have had enough of it." [¶¶ 26 to 27, 155]

At 1:43 pm the Manager replied, apologising for the rostered shift and confirming it was "an oversight", and asking "Would you like me to roster you off tomorrow Tuesday 8th 0700-1500 to give you the required rest period". [¶¶ 156 to 157]

At 3:13 pm the Director replied. [¶ 158]

On 8 August 2023 at 4:15 pm the Director wrote regarding an additional shift to replace the shift not worked due to a rostering error. [¶ 28]

On 29 August 2023 she sent HR Policy E12, and on 4 September 2023 the Appellant replied. [¶¶ 160 to 161]

On 31 August 2023 he provided a written application headed "Request to Increase Working Hours to Full Time Rotational Roster", expressing willingness to take on additional night shifts. [¶¶ 30 to 31]

On 4 September 2023 he confirmed that he was able and willing to work any roster presented to him including the current schedule with full 24-hour availability, that he had no issues with shift work, and attached a draft roster spreadsheet. [¶¶ 33 to 35]

On 27 September 2023 the Manager wrote with the subject "Approved - Permanent Full Time FTE", stating she was happy for him to commence full-time hours from 16 October 2023. [¶¶ 37 to 38]

On 8 August 2023 he had confirmed that he would work the replacement shift and was looking to pick up another. [¶ 29] On 8 September 2023 the Director wrote that she was "glad to hear things seem to be going well with Chloe" and glad that he had applied for the additional shifts through the recent EOI. [¶ 36]

1(d) Notification of variable office hours (23 August 2023 to 17 May 2024). The position reports to the Switchboard Manager, and the role description names that person as the contact for the position. [¶¶ 3 to 4]

On 23 August 2023 at 2:18 pm she wrote to Logan Switch with the subject "What's Chloe's Hours?!": "I am fulltime and work flex hours - You may have noticed recently I am starting at different times in the morning and finish at different times in the afternoon", and "My hours would range from starting between 6-9am and finishing 2-5pm." [¶¶ 70 to 72]

The Respondent does not allege that fixed office hours were stated to the Switchboard staff at any time between 23 August 2023 and 17 May 2024. [¶ 73] That is approximately nine months.

1(e) Pandemic leave submissions (20 February to 1 March 2024). On 20 February 2024 the Appellant messaged the Manager that he had tested positive for COVID-19. She "requested that you send it to her so she could submit it in MyHR". [¶ 136] She had said he was eligible for up to five days' paid leave on photo evidence of the test. He sent it to her as she asked. She then asked him to submit the Special Pandemic Leave request through myHR himself and attach a statutory declaration or photo evidence of his test. [¶ 132] The myHR submissions report records the initiator of that request as the Appellant [¶¶ 140 to 141], and the Respondent does not allege that any other Switchboard employee was required to submit such a request personally through myHR in February 2024. [¶ 142] The leave request history for Process Reference 15480560 records him creating a draft at 11:24:48 that morning and submitting the request at 11:41:27. [¶¶ 114 to 115] So the material was provided four times in all: once to the Manager, as requested, and then three times through myHR, once that morning and, after two declines, twice more.

It records the request being declined at 12:23:43 on 21 February 2024; submitted again at 06:07:32 on 28 February; declined again at 09:06:07 on 29 February; submitted a third time at 11:07:28 that morning; and approved at 11:21:03. [¶¶ 116 to 120]

It records the Appellant as having submitted the request on three occasions and the Manager as having declined it on two, and records the Director as approving it at 16:05:08 on 1 March 2024. [¶¶ 121 to 123] The approval followed the third submission by thirteen minutes and thirty-five seconds; the interval from draft to final approval was ten days, four hours, forty minutes and twenty seconds. [¶¶ 124 to 125] Two reasons for the declines appear on the record and they are not the same. The review decision records that on 21 February the Manager messaged that she had to decline because the minimum period for COVID leave was five days for a positive staff member, that she noted the statutory declaration provided was sufficient evidence, and that she asked him to resubmit his leave days. [¶¶ 137 to 138]

It records the Appellant messaging that he was confused about why his amended request was declined, and that he had done everything as required. [¶ 139]

The Respondent pleads that she declined the application because on her assessment the required statutory declaration was not attached; that "a review indicates that in fact, the attachments were present on the appellant's submission"; and that this was human error on the background of high work demands. [¶¶ 133 to 135]

So the request was made on 20 February 2024 and finally approved on 1 March 2024: ten days, four hours, forty minutes and twenty seconds from draft to final approval, with two declines and three submissions in between. The third submission was approved in thirteen minutes and thirty-five seconds. [¶¶ 114 to 125]

Two matters are repeated here from Stressors 2 and 3. The myHR submissions report records the Manager as the initiator of each of the five Attendance Variation and Allowance Claims made for the Appellant between 1 February and 31 May 2024, and the Appellant as the initiator of none [¶¶ 198 to 201]; the special pandemic leave request was the submission he was required to make himself [¶¶ 132, 140 to 142]. The Respondent pleads human error in the handling of that request [¶¶ 133 to 135] and human error in the rostering of 17 and 18 March 2024 [¶ 226], events less than a month apart.

1(f) After hours on call process changed (15 April 2024). On 15 April 2024 at 12:39 pm the Switchboard Manager wrote to Logan Switch, copied to the Director and the Switchboard staff, with the subject "Afterhours Oncall Process - Switchboard". [¶ 49] That email stated that the process during office hours remained the same and that staff were to contact her through switch, the office or her mobile, and: "You will see that Ellen and myself have added Afterhours on call, the days that are highlighted in purple show who is on call after hours." [¶¶ 50, 52] It stated: "This new process is effective from today." [¶ 51]

She signed it as "A/Switchboard Manager". [¶ 53]

1(g) Data entry process changed (19 April 2024). The review decision records that on 19 April 2024 she emailed the team stating that due to errors being made with respect to data entry a new process was to be followed which included more checks to ensure accuracy of data entry. [¶ 54]

1(h) Misdirected calls and the MASPER response (2 to 9 May 2024). A key responsibility of the position is to participate in the Emergency Response process by receiving emergency response notifications and distributing them to the appropriate response groups, dependent on the category of emergency, as per emergency code procedures, strictly adhering to protocols and timeframes; to maintain call queues to a minimum at all times; and to operate under pressure, particularly where high volume call traffic is concerned. [¶¶ 5, 8, 10]

In April 2025 the console took between 269 and 444 calls on an eight-hour shift (Stressor 3(b), the ordinary-call-load paragraph below); the Appellant's evidence is that the load in May 2024 was of the same order. Emergency codes were received and paged through it throughout the day (Stressor 3(b)).

On 3 May 2024 at 3:06 pm the MASPER Registrar wrote to the Switchboard Manager with the subject "Switchboard issues 2nd and 3rd May 2024", listing five occasions on 2 and 3 May. [¶¶ 56 to 57] Of the occasion at 14:46 on 3 May the email records: "MET call team called x5290 asking where MET call was located 'VHUB' - switchboard could not tell them where VHUB was. Had to be redirected by MASPER". [¶ 58]

On 8 May at 5:28 pm she wrote again, listing four further occasions, recording of those on 8 May that a call was "incorrectly put through to MASPER" on each occasion. [¶¶ 59 to 61] Those nine occasions fell while the arrangement notified on 15 April 2024 was in force. [¶ 68]

The first reply was sent on 9 May at 9:20 am, and asked Dr Wong, to whom the reports had been copied, to confirm her business hours because they were "not provided on the rosters", noting that her dect phone was "currently switched off". [¶ 65]

That reply was sent five days, eighteen hours and fourteen minutes after the first email. [¶¶ 62 to 64]

At 10:15 am the same day the Switchboard Manager wrote to Logan Switch introducing a new process and referring to "many ongoing issues raised by the MASPER and the medical department about calls being transferred to the wrong medical teams". [¶¶ 66 to 67]

The Respondent does not allege that any communication was sent to Logan Switch or to the Switchboard staff concerning those occasions at any time after 3 May 2024 and before 10:15 am on 9 May 2024. [¶ 69]

So the sequence at the console was: the first report on 3 May 2024, a second on 8 May (the fifth calendar day after the first), nine occasions in all, a first reply after five days, eighteen hours and fourteen minutes, and a new process communicated to Switchboard staff at 10:15 am on 9 May (the sixth day). [¶¶ 56 to 69]

1(i) The complaint of 13 May 2024 - chronology only (13 May 2024). On 13 May 2024 the Appellant lodged a corrupt conduct complaint regarding clinical risks. The Ethical Standards Unit formally determined this constituted a Public Interest Disclosure (Admitted Fact: Form 24, para 20, admitted 18 February 2026). The content is not set out. It is pleaded for chronology only; no cause of the injury and no motive is alleged in connection with it.

1(j) On-call roster and absence notification instructions - context only (13 to 15 May 2024). On 13 May 2024 at 4:29 pm the on-call roster for the fortnight beginning that day was circulated to the Switchboard team. [¶¶ 111 to 112]

From 14 May 2024 the process for notifying unavailability for a rostered shift was also directed through the Switchboard Manager, both emails identifying the Switchboard as a means of contacting her. [¶¶ 162 to 165]

The Respondent does not allege that the Appellant was informed before 14 May 2024 that any other means of notification was required. [¶ 166]

1(k) Respiratory directory error and requests for correction (15 to 20 May 2024). On 15 May 2024 at 11:47 am an Administration Officer of the Integrated Respiratory Service wrote to Logan Switch, marked of High importance: "Could you please amend your number registry/directory to show #8768 belongs to the Integrated Respiratory Service... We are not Respiratory Medical OPD and we do not have any doctors working out of this area." [¶ 89]

The Respondent does not allege that any response was made to that email before 20 May 2024. [¶ 90]

On 20 May at 11:03 am she wrote again, marked of High importance: "Just a courtesy reminder, we continue to get calls put through to us for Respiratory Medical Outpatients... we can not help patients or other clinical staff with OPD issues." [¶¶ 91 to 92]

The Respondent does not allege that any response was made to that email before 2:05 pm that day. [¶ 93]

At 2:05 pm, from the Logan Switch account and within the office hours stated to all Switchboard staff on 17 May 2024, the Appellant wrote identifying the cause as modifications made to the document "Outpatients Department - Clinic contact Details" on 22 February 2024, and recommending a modification and review of that document. [¶¶ 94 to 96] That email was sent three hours and two minutes after the Service's second email, and five days, two hours and eighteen minutes after its first. [¶¶ 97 to 98]

The Respondent does not allege that the Appellant was rostered to work after 2:05 pm that day. [¶ 99] The reply at 4:30 pm stated: "Thank you for bringing this to my attention however this task was being actioned. I had discussed with Richard this morning about the update of outpatients respiratory/medical." [¶¶ 100, 104] It was sent two hours and twenty-five minutes after the Appellant's email, and two hours after the conclusion of the office hours stated to all Switchboard staff on 17 May 2024. [¶¶ 102 to 103]

The Respondent's amended List of Documents records no document created on 20 May 2024 recording that discussion, and no communication sent by either named person to the Service on that day; and the Respondent does not allege that the other named person communicated with the Service on that day. [¶¶ 105 to 108]

The Respondent does not allege that the contact-details document was amended on or before 20 May 2024, or that the Switchboard staff were notified of the modifications made to it on 22 February 2024. [¶¶ 109 to 110]

So the sequence was: the Service's first email on 15 May 2024; its second on 20 May, the fifth calendar day; the Appellant's escalation five days, two hours and eighteen minutes after the first email and three hours and two minutes after the second; and the reply two hours and twenty-five minutes after that, two hours after the stated office hours had ended. In that time the Respondent alleges no response to the Service, no amendment of the document, and no notice to the staff of the change made to it on 22 February 2024. [¶¶ 90, 93, 97 to 98, 102 to 103, 105 to 110]

On 20 May 2024 at 4:30 pm the Switchboard Manager wrote: "Taking note of your recommendation, we can also put the updated procedures out to the team for consultation before implementing." [¶ 101]

1(i) Office hours request and the retraction response (15 to 21 May 2024). On 15 May 2024 at 1:15 pm the Appellant wrote to the Manager and Logan Switch, copied to the Switchboard staff, the Director and LBH_HR, asking that she share her office hours so the department could be aware of her regular schedule, noting inconsistency in arrival and departure times, and asking that "any directives and changes within the department is made in consultation with the team". [¶¶ 74 to 75]

At 6:23 pm the Director replied, copied to the Manager and marked of High importance, stating that his email "did not demonstrate our iCARE2 value of Respect and did not comply with our Code of conduct", and asked him to retract it. [¶¶ 76 to 77]

At 7:09 pm the Appellant replied that "Requesting clarity on business hours is a reasonable question". [¶ 78] The Respondent's outline of Ms Taylor's evidence, served 24 September 2026, states that, acting on HR's advice, she removed the email of 15 May 2024 from the shared Switchboard inbox.

On 17 May 2024 at 9:30 am the Manager wrote to the whole department, headed "Switchboard Manager - On call and Hours", stating that her hours "can vary", that "I will be in later between 0800-830am. Otherwise my hours are from 06:30-14:30", that "I will not be in the office until around 11am on Monday 20.05.2024", and that "Moving forward so communication is clear for the team, I will be sending an email to switch to advise of any change to my office hours for the week". [¶¶ 81 to 84, 113] Those stated office hours concluded at 2:30 pm. [¶ 88]

On 21 May 2024 at 2:53 pm the Director replied that she would follow up on the issues raised, and asked "can I ask was there one or more particular changes and/or directives where you had concerns about consultation and communication?" [¶¶ 79 to 80]

So the request for the Manager's office hours was made at 1:15 pm on 15 May 2024. The request to retract it came five hours and eight minutes later, at 6:23 pm. The hours were stated to the whole department one day, twenty hours and fifteen minutes after the request, at 9:30 am on 17 May. The Director's question about which changes had concerned him came six days, one hour and thirty-eight minutes after the request, at 2:53 pm on 21 May. Before the request, no fixed hours had been stated for approximately nine months. [¶¶ 73 to 84]

Supporting records and overlap with the other themes

The Manager's notice of 18 June 2024. On 18 June 2024 at 8:58 am the Manager wrote to Logan Switch: "I am taking today off, and hopefully will be back tomorrow", and "I am sorry I haven't been there for you all over the past week." [¶¶ 85 to 87]

By letter of 5 June 2026, reference K-LM26/729, signed by the Chief Executive and addressed to the Commission, Metro South Health stated that the requested fatigue risk management documents "do not exist", that mandatory Fatigue Risk Management System training "only applies to health practitioners and clinical assistants" and that Switchboard staff are non-clinical. [¶¶ 263, 265] It stated that "The implementation of fatigue risk management assessment at Switchboard Logan Hospital occurred after 30 June 2024". [¶ 264] It stated that "there have been no 'consequential' changes to operating procedures over the period requested". [¶ 266] It stated that "All employee complaints relating to Logan Hospital Switchboard operational errors are made directly to the Line Manager of Switch Board and managed solely via email or verbally with the complainant." [¶ 267] It stated that "a spreadsheet of recorded MET calls is available for the period 17-18 March 2024". [¶ 268]

The Respondent does not allege that any fatigue risk assessment was conducted, that any fatigue risk management training was provided, that fatigue risk management assessment was implemented at the Switchboard, or that any change was made to the operating procedures of the Switchboard as a consequence of any employee complaint, at any time before 30 June 2024. [¶¶ 269 to 272]

Payroll's direction of 3 May 2024 to correct four fortnights had not been actioned on 13 May [¶¶ 192 to 193] and was still awaited on 21 May [¶¶ 194 to 195]; that sequence is pleaded at Stressor 2(h), 2(i), 2(l) and 2(m) and is repeated here as part of the conditions in which the work was carried on.

The rostering concern raised in August 2023 recurred, and the employer's own documents record it. On 26 April 2024 the Director wrote that the Manager "was working to fix this error" [¶¶ 211 to 212]; on 10 May 2024 she wrote to Human Resources that the Appellant's concern was about how the Manager was rostering the team and its effect on staff fatigue [¶ 218], acknowledged "a few rostering errors made by Chloe with regards to Cory's line in past rosters" [¶ 220], and stated that his roster would not be considered [¶ 221]; and on 18 February 2026 the Respondent admitted that the Director stated on 7 August 2023 that there had been a rostering error that was accidentally made [¶ 287]. These facts are pleaded at Stressor 3(a) and 3(e) to 3(g) and are repeated here because they bear on this stressor.

Context and the Respondent's pleaded position. The Respondent does not allege that the Appellant was subject to any disciplinary process, or that his work performance was the subject of any formal performance management process, before 18 June 2024, nor does it describe any communication with him before that date as a warning. [¶¶ 300 to 302] Paragraph 27 of its amended statement of facts and contentions does not identify the management action relied upon. [¶ 303] The matters at 1(a), 1(c), 1(e), 1(i) and 1(j), and at paragraph 3 below, are pleaded as part of the sequence of matters the Appellant raised and the responses he received. They are not relied on as separate causes within Stressor 1. Where an admitted fact from that sequence is repeated under Stressor 2 or Stressor 3, it is relied on there only for the purpose stated under that stressor. The circumstances relied on as causes in Stressor 1 are those at 1(b), 1(d), 1(f) to 1(h), 1(k) and 1(l), read with the opening duties and standards and the supporting records.

The medical link - Stressor 1. Tab M1 records the 28 June 2024 presentation as "stress at work" and "upset by people not following rules". Tab M4 records the treating psychiatrist's later account of origin as workplace stress involving management and rostering. Those records are relied on as evidence, made at the time and by the treating doctors, of what was reported, the diagnosis and the effect. They are not relied on to prove that the events in Stressor 1 happened. Those events are proved, so far as they are relied on here, by the admitted facts and the employer documents identified.

STRESSOR 2 - REMUNERATION

Scope of Stressor 2. Particulars 2(a) to 2(i) and 2(l) to 2(n) identify the entitlement and correction events before the certified injury date. Particular 2(j) records a completed claim in the same period, and particular 2(k) the related HR follow-up; both are context. Particular 2(o) records a later correction of the earlier error, not an additional cause arising in 2025.

The basis of the entitlement. The position is a continuous shift working role over the full 24-hour period, seven days a week, and under mandatory requirements the occupant must be able to work a roster which covers multiple shifts over a 24/7 period. [¶¶ 2, 13] The function is admitted to be critical to patient safety [¶ 289]. The Appellant moved to full-time hours from 16 October 2023 on the Manager's written approval [¶¶ 37 to 38], and the review decision records the employer confirming the change to his employment contract and working hours [¶¶ 18 to 20]; the contracted 76 hours a fortnight against which Payroll measured the corrections is that arrangement [¶ 189]. In August 2023 a shift not worked because of a rostering error had been replaced by an additional shift [¶¶ 28 to 29]. These facts are pleaded at Stressors 1(c) and 3(a) and are repeated here.

Who submitted the recorded corrections. The myHR submissions report records seven submissions for the Appellant in the period 1 February to 31 May 2024, of which five were Attendance Variation and Allowance Claims. [¶¶ 198 to 199]

It records the initiator of each of those five as the Manager, and the initiator of no such claim in that period as the Appellant. [¶¶ 200 to 201]

2(a) Seven hours recorded instead of eight (9 February 2024). The shift of 9 February 2024 was recorded as seven hours instead of eight, so the fortnight commencing 5 February 2024 was under the contracted hours and was topped up from the Appellant's RDO balance. [¶¶ 185 to 186]

2(b) Processing of paid pandemic leave (20 February to 1 March 2024). The power to approve or not approve paid Special Pandemic Leave for the COVID-19 event was sub-delegated by the Acting Health Service Chief Executive to a Band 9 delegate, and the Instrument states that the power could not be further sub-delegated. [¶¶ 127 to 128] On 20 February 2024 the Manager asked the Appellant to send his material to her so she could submit it in myHR [¶ 136]; he did so; she then asked him to submit it through myHR himself [¶ 132], and the leave history records him creating the draft and submitting the request that morning. [¶¶ 114 to 115] The history records the request being declined on 21 February, resubmitted on 28 February, declined again on 29 February, submitted a third time at 11:07:28 that day and approved at 11:21:03. [¶¶ 116 to 120] It records the Appellant as making three submissions, the Manager as declining twice, the Manager as "Reviewer" and the Director as "Manager", with the Director recording final approval on 1 March 2024. [¶¶ 121 to 123, 126] The review decision records that on the first decline the stated reason was the minimum period for COVID leave while the statutory declaration was said to be sufficient; it records the Appellant saying he was confused by the second decline and had done everything required. [¶¶ 137 to 139] The Respondent's pleading states that the later decline was because the statutory declaration was assessed as not attached, but that review showed the attachments were in fact present, and pleads human error against a background of high work demands. [¶¶ 133 to 135] The Respondent does not allege that any other Switchboard employee was required personally to submit such a request through myHR in February 2024. [¶ 142] This sequence is repeated on purpose from Stressor 1(e). In Stressor 2 it is relied on as the handling of a paid employment entitlement: an entitlement that took three submissions and a decision process the Appellant could not complete by himself. It is not relied on as a separate injury.

2(c) Second shift recorded as seven hours (28 February 2024). The shift of 28 February 2024 was recorded as seven hours instead of eight, and the fortnight commencing 19 February 2024 was topped up from the Appellant's RDO balance. [¶ 187]

2(d) Overtime recorded as ordinary shifts (fortnight beginning 18 March 2024). The fortnight commencing 18 March 2024, which contained the shift of 18 March, was recorded with too many ordinary shifts, reducing wages, because one of the shifts needed to be overtime. [¶ 188]

2(e) Public holiday claim submitted late (30 March 2024). The public holiday of 30 March 2024 was, on Payroll's later account, "submitted late".

2(f) Hours above the contracted fortnight (fortnight beginning 1 April 2024). The fortnight commencing 1 April 2024 was over the contracted 76 hours, again requiring overtime. [¶ 189]

2(g) Pay enquiry and public holiday review (8 April to 1 May 2024). The Appellant raised the pay for the period 8 to 31 March 2024 with the Manager on 8 April 2024. [¶ 242] On 9 April she replied that she had escalated the fatigue enquiry to Human Resources. [¶ 244] On 24 April he wrote that it had been more than two weeks without any response or overtime payment. [¶ 246] On 1 May she replied that the fatigue payment for 18 March would not be processed, and that payroll was still reviewing his public holiday entitlements. [¶¶ 247, 255]

Public holidays on which the Appellant was not required to work. The review decision records that section 23 of the Award provides that where an employee in receipt of the additional week's leave prescribed by clause 19.2(a) is rostered off on Easter Saturday, Easter Sunday, Show Day or Labour Day, that employee is to be paid an additional day's wage or granted a day's holiday in lieu. [¶ 241] As at 1 May 2024 payroll was "still reviewing your entitlements regarding public holidays not required arising while you were a part-time employee". [¶ 255] The Manager confirmed "that since your commencement of full-time employment, all payments for public holidays not required had been processed". [¶ 256]

2(h) Payroll directed four fortnight corrections (3 May 2024). The Respondent's disclosure contains an email thread with the subject "Corey Shepherd 388372 Pay issues", the first message being from PayrollMetroSouth on 3 May 2024 to the Manager, copying the Appellant, bearing the signature block of a Client Service Officer of the Metro South Payroll Team. [¶¶ 182 to 184] That message identifies four fortnights requiring correction and states: "Please submit an AVAC to correct these shifts for each fortnight so Cory is paid corrected and his RDO balance will then be amended." [¶¶ 185 to 190]

2(i) Follow-up found no correction (10 to 13 May 2024). On 10 May 2024 at 2:21 pm the Appellant wrote to PayrollMetroSouth and Ms Grant. [¶ 191]

On 13 May 2024 at 8:21 am Payroll replied to the Appellant: "I cannot see that any of the issues below have been corrected. Please speak to your Line Manager to have them corrected with an AVAC submitted through My HR". [¶¶ 192 to 193]

2(j) A completed AVAC recorded - context only (15 to 16 May 2024). The myHR submissions report records a claim with process number 16328886 submitted on 15 May 2024 and completed the next day. [¶ 202]

2(k) HR follow-up about rostering (20 May 2024). On 20 May 2024 at 4:07 pm the Director wrote to LBH_HR: "I am just following up on this query I raised a little while ago in relation to a email I had received from a staff member about our rostering practices. Thus if someone might be able to give me a call regarding these staff concerns, that would be great." [¶ 222] That email attached the document "qh-gdl-401-3.3", the Queensland Health Fatigue Risk Management Systems Implementation Guideline. [¶ 223]

2(l) Manager awaited Payroll confirmation (21 May 2024). On 21 May 2024 at 12:33 pm the Manager wrote that she was "still I am waiting payroll confirmation". [¶¶ 194 to 195]

2(m) Validation request and partly completed AVAC (28 to 30 May 2024). On 28 May 2024 at 8:36 am she wrote: "Please find attached Validation of claims older than 3 months, please sign and return to be as soon as possible so I can escalate for delegate approval." [¶¶ 196 to 197]

The myHR submissions report records a claim with process number 16450619 submitted on 28 May 2024, with an effective date of 30 March 2024 and a processing date of 30 May 2024, with a status of "Part Completed", and records the status of every other such claim in the period as "Completed". [¶¶ 203 to 204]

The myHR report records the process numbers and submission dates in the period, and those process numbers increase as submission dates increase. [¶¶ 205 to 206]

2(n) Three corrections reached a payslip (5 June 2024). When each correction was made is stated by Payroll itself, in its email to the Respondent of 11 July 2025 and the payslips attached to it, disclosed by the Respondent in July 2025 (Disclosure from witness conferencing, Queensland Health Payroll, pp 2 to 3 and 19 to 24). The shifts of 28 February, 30 March and 1 April 2024 were adjusted on Payslip 06, for the pay period 13 to 26 May 2024, pay date 5 June 2024. That pay date was two days after the Appellant's last shift, after he had stopped attending work, thirteen days before the certified date of injury, and 33 days after Payroll's direction. On Payroll's account the 28 February adjustment was made by reversing the hours top-up and replacing it with an adjustment "which is leave without pay for the hour on the 28/02/2024 shift"; the 30 March entry was "amended to 'PubHolLv-DayPayforOffDay' (public holiday allowance)"; and the 1 April shift was amended to overtime.

2(o) Later correction of the 9 February shift (30 June to 2 July 2025). The shift of 9 February 2024 was not corrected on Payslip 06, pay date 5 June 2024. On Payroll's account it "was amended with an exemption for the Validation of Claims form on the 30.06.2025 and is on Payslip 07", for the pay period 9 to 22 June 2025, pay date 2 July 2025. That was 425 days after Payroll's direction of 3 May 2024 (the amendment itself being dated 30 June 2025); after the date of injury; after the Respondent's review decision of 24 October 2024; and after this appeal was commenced. It was made with an exemption from the form the Manager had asked the Appellant to sign on 28 May 2024. [¶¶ 196 to 197] The Respondent's pleading describes the pay issues as "remedied in a timely manner" [¶ 207], and its outlines of evidence describe them as "corrected" (Ms Wright's outline, served 24 September 2026), without stating these dates.

The correction chronology read together

On 3 May Payroll identified all four fortnights and directed the Manager to submit an AVAC for each. [¶¶ 185 to 190] On 10 May the Appellant wrote to Payroll. [¶ 191] On 13 May Payroll replied that none of the issues had been corrected and that the Line Manager had to submit the AVAC. [¶¶ 192 to 193] On 21 May the Manager wrote that she was still waiting for payroll confirmation and: "I will let you know PRN once it has been submitted." [¶ 195] On 28 May she sent the Validation of Claims form for signature and submitted the AVAC; it was processed on 30 May as "Part Completed". [¶¶ 196 to 197, 203, 209] He was not told that it had been submitted, or its process reference. His last day of attendance for a rostered shift was 3 June 2024; he was on leave from 7 to 12 June 2024; the date of injury recorded on the work capacity certificates is 18 June 2024 (Part B.1.4 to 1.5).

So the pay sequence was this. The Appellant raised the pay for 8 to 31 March on 8 April 2024; the reply came the next day; his follow-up came fifteen days after that, on 24 April; and the refusal came seven days later, on 1 May. Two days after the refusal, on 3 May, Payroll identified all four fortnights and directed the Manager to submit an AVAC for each. From that direction: the Appellant wrote to Payroll after seven days (10 May); Payroll wrote that nothing had been corrected after ten days (13 May); the Manager wrote that she was still waiting after eighteen days (21 May); she submitted the AVAC after 25 days (28 May); it was processed "Part Completed" two days after that (30 May); and three of the four corrections reached a payslip after 33 days (pay date 5 June 2024), two days after his last shift. From the shifts themselves to the payslip that adjusted them: 98 days for 28 February, 67 days for 30 March, 65 days for 1 April. The 9 February shift was corrected 423 days after Payroll's direction (30 June 2025) and reached a payslip after 425 days (2 July 2025): 509 days after the shift was worked. In all that time the Respondent alleges no step other than those listed. [¶¶ 185 to 197, 203, 209, 242 to 247] The enquiry of 8 April 2024 was also the fatigue enquiry pleaded at Stressor 3(d); no further response to it is alleged between the acknowledgment on 9 April and the substantive reply on 1 May, and it was answered after 23 days [¶¶ 249 to 250].

The Respondent's pleaded answer and the record

The Respondent pleads that "any discrepancies or errors were remedied in a timely manner" and that "there are no outstanding underpayments for the appellant", and that the Manager submitted the AVAC on 28 May 2024. [¶¶ 207 to 209]

Neither AVAC PRN 15397775 nor AVAC PRN 15605601 appears in the myHR submissions report. [¶ 210]

On 18 February 2026 the Respondent admitted the Payroll instruction of 3 May 2024, the fact that the Manager submitted the AVAC on 28 May 2024, and the email of 21 May 2024 stating that she was waiting for payroll confirmation. [¶¶ 296 to 298]

The Respondent's pleaded answer to this stressor. The Respondent does not allege that the Appellant was subject to any disciplinary process, or that his work performance was the subject of any formal performance management process, before 18 June 2024, nor does it describe any communication with him before that date as a warning. [¶¶ 300 to 302] Paragraph 27 of its amended statement of facts and contentions, which contends that any management action was reasonable management action taken in a reasonable way, does not identify, by particular, date, document or cross-reference, the management action relied upon. [¶¶ 238, 303] The employer's letter of 5 June 2026 states that there were no consequential changes to operating procedures over the period requested [¶ 266] and how employee complaints were managed [¶ 267], and the Respondent does not allege any change to the operating procedures of the Switchboard as a consequence of any employee complaint before 30 June 2024 [¶ 272]; those facts are pleaded at Stressors 1, supporting records and 3, supporting fatigue records and are repeated here.

The medical link - Stressor 2. Tab M1 records the 1 July 2024 report of "work stress" and that pay was withheld at times and overtime was not processed. Tab M4 records the treating psychiatrist's later history

that pay was "withheld or delayed for up to five months at a time", as a source of financial stress. The medical records are relied on for what was reported and for the clinical course. The pay events themselves are proved by the admitted payroll, myHR and pleading facts set out above.

STRESSOR 3 - THE ROSTER AND FATIGUE

Scope of Stressor 3. The dated particulars below identify the roster, recovery and subsequent handling of fatigue concerns. The 2020 agreement is a matter bearing on the applicable minimum break. Its execution is not pleaded as a separate injuring event. The seven-hour roster is relied on even if the eight-hour agreement applied. The causal particulars are 3(b) to 3(h). Particular 3(a) is pleaded as context, not as a separate cause.

The standard, as admitted. The review decision records that the employer's own response included an extract of the Award stating that employees must be provided with a break of not less than ten hours between the termination of one shift and the commencement of another, and that eight hours applied instead of ten only in specific circumstances. [¶ 257]

The Award itself, clause 15.1(d), provides: "Employees must be allowed a break of not less than 10 hours between the termination of one shift and the commencement of another shift, except that 8 hours applies instead of the 10 hours in any of the following circumstances: (i) for the purposes of changing shift rosters by shift workers who rotate from one shift to another; or (ii) in any other case agreed upon between the employer and the employee concerned." Clause 18.10, on which the Respondent relies at paragraph 24(b) of its statement [¶ 237], is headed "Fatigue leave/rest period after overtime" and applies where overtime is worked. [Hospital and Health Service General Employees (Queensland Health) Award - State 2015]

On 18 February 2026 the Respondent admitted that "The Employer's 'Fatigue Risk Management Policy' and the relevant Award require a minimum break of 10 hours between shifts, or 8 hours by written agreement". [¶ 285] It denied a further paragraph of that notice on the ground that in June 2020 the Appellant signed an agreement allowing an 8-hour break between shifts. [¶ 286]

The agreement of 17 June 2020, and its scope. On 17 June 2020 the Appellant signed an agreement allowing an 8-hour break between shifts. [¶ 17]

The change to part-time. Since 17 June 2020 there has been a change to the Appellant's employment contract and adjustments in his working hours, as the review decision records the employer confirming. [¶ 20] He later became a part-time employee. In August 2023 he sought additional hours under clause 11.7 of the applicable agreement, headed "Additional Permanent Hours for Part-time Employees" [¶¶ 26 to 27], and Payroll was, as at 1 May 2024, still reviewing his entitlements "regarding public holidays not required arising while you were a part-time employee". [¶ 255]

The change to full-time. On 31 August 2023 he applied in writing to increase his hours to a full-time rotational roster. [¶ 30] On 27 September 2023 the Manager approved his application for "Permanent Fulltime hours", with full-time hours to commence from 16 October 2023. [¶¶ 37 to 38] The review decision records his submission that the 8-hour agreement "was no longer fit for purpose since your transition from casual employment to full-time" [¶ 18], and that "since your commencement, you have signed new terms and working arrangements as a full-time employee". [¶ 19]

The emails. On 1 May 2024 the Manager wrote that the request for fatigue payment for 18 March 2024 would not be processed "due to the existing 8-hour agreement signed by you on 17 June 2020", noting that he was able to terminate the agreement going forward; that email is pleaded at 3(d) below. [¶¶ 247 to 248] The review decision records the employer's response of 6 September 2024 as confirming "you did not formally rescind the 8-hour agreement signed by you on 17 June 2020 and the change to your employment contract and adjustments in your working hours did not automatically invalidate the agreement". [¶ 252] By email of 7 July 2026 a Senior Consultant, Human Resources wrote that the rostering "currently provides more than" the required break, and acknowledged the 8-hour agreement signed on 17 June 2020 "which allows you to work with only an 8 hour break, however this is only applied where staff initiated shift swaps have occurred". [¶¶ 224 to 225]

The Manager and the agreement. The review decision records that Ms Sandra Johnstone was the Switchboard Line Manager as at 8 December 2021, that Ms Danielle Cook was the "previous Switchboard Manager", and that "Ms Taylor was subsequently appointed to the Switchboard Manager role". [¶¶ 23 to 24] The Respondent does not allege that Ms Taylor was the Switchboard Manager on 17 June 2020. [¶ 25]

The rest of the record on the agreement. The Respondent does not allege that the agreement was reviewed, re-executed or re-confirmed at any time between 17 June 2020 and 18 March 2024 [¶ 21], or that the Appellant was informed, at any time before 1 May 2024, that it could be terminated by him. [¶ 22] The review decision found "uncertainty between you and the employer regarding whether the 8-hour agreement

continued to apply" [¶ 254], and found the rostering of the two shifts to be "in direct contradiction to the award and the 8-hour agreement". [¶ 260]

The Appellant's contention on the agreement. The Appellant disputes the agreement's application to these shifts and relies on the admitted facts above and the HR account of 7 July 2026. Its application to March 2024 is a contention for determination. Alternatively, even if the agreement applied in March 2024, a seven-hour interval was below eight hours.

The role description states, under mandatory requirements, that the position is a continuous shift working role and that the occupant must be able to work a roster which covers multiple shifts over a 24/7 period. [¶ 13]

The emergency workload on those shifts. Metro South Health has stated in writing that "a spreadsheet of recorded MET calls is available for the period 17-18 March 2024". [¶ 268] A key responsibility of the position is to participate in the Emergency Response process by receiving emergency response notifications and distributing them to the appropriate response groups, dependent on the category of emergency, as per emergency code procedures, strictly adhering to protocols and timeframes; and to support the allocation, coordination and fault repair of all pager units for the hospital. [¶¶ 7 to 8] The Switchboard staff are, on the employer's own statement, non-clinical. [¶ 265]

The codes. Each emergency code at Logan Hospital is received at the Switchboard, categorised by the operator by the type of emergency, and paged to the response group for that category within the time the procedure allows. [¶ 8] A Code Blue is the call for a medical emergency in which a patient has suffered, or is about to suffer, cardiac or respiratory arrest, and brings the resuscitation team. A MET call is the call for the Medical Emergency Team to a patient whose observations have deteriorated to the hospital's escalation threshold; a Neonatal MET call and a Paediatric MET call are the same call for a newborn and for a child. A Code Grey is the call for a security response to threatening or aggressive behaviour; a Code Black is the call for a security response to a personal threat. The colour codes follow the Australian Standard for emergency control in health care facilities, AS 4083. Queensland Health's own Code Blue procedure for the Royal Brisbane and Women's Hospital (Procedure 001443, version 5, effective April 2020) states that "there are eight different types of Code Blue", including Medical Emergency, Cardiac Arrest, Neonatal Emergency, and Paediatric Emergency or Paediatric Cardiac Arrest; that "Switchboard Services will activate one or multiple teams to provide a Code Blue response"; and that the response team "will be notified by a paging system from Switchboard Services... identifying the type of Code Blue and the location of the emergency". The Logan Hospital emergency code procedures referred to in the role description [¶ 8] are the equivalent document for Logan Hospital (Schedule, Part 2).

The employer's own fatigue policy. The review decision records that the policy regarding fatigue leave following weekends and rostered days off is dated June 2020. [¶ 239]

It records that "The policy notes that many problems can be overcome regarding fatigue leave by either not rostering the employee who is on call for the first shift of the following day and not rostering on call an officer who is rostered for the first shift on the following day." [¶ 240]

3(a) Earlier roster error and proposed roster - context only (7 August to 4 September 2023). On 7 August 2023 the Manager acknowledged "an oversight" in the roster and offered to roster the Appellant off the following day "to give you the required rest period". [¶¶ 156 to 157]

On 18 February 2026 the Respondent admitted that the Director stated in an email dated 7 August 2023 that there had been a rostering error that was accidentally made. [¶ 287] The Appellant applied in writing to increase his hours to a full-time rotational roster, confirmed he was able and willing to work any roster presented to him, expressed willingness to take additional night shifts, and provided a draft roster spreadsheet. [¶¶ 30 to 31, 33, 35]

On 8 August 2023 the Director wrote regarding an additional shift to replace the shift not worked due to a rostering error. [¶ 28]

3(b) Consecutive shifts and the seven-hour break (17 to 18 March 2024). The Appellant was rostered to finish at 23:00 on 17 March 2024 and to commence at 06:00 on 18 March 2024, a break of seven hours. [¶ 284] The shift commencing on 18 March fell on a Monday, the first shift of the working week. [¶ 227] Payroll later recorded that the fortnight commencing 18 March 2024 had too many ordinary shifts and that one of the shifts needed to be overtime [¶ 188], as pleaded at Stressor 2(d). The break was rostered in a function the Respondent admits is critical to patient safety [¶ 289].

The Respondent does not allege that those consecutive shifts arose from a staff-initiated shift swap. [¶ 234] So, on the admitted record: the minimum break was ten hours, or eight by written agreement [¶ 285]; on 7 July 2026 the employer's Human Resources wrote that the 8-hour agreement "is only applied where staff initiated shift swaps have occurred" [¶¶ 224 to 225]; no swap is alleged [¶ 234]; the break was seven hours [¶ 284]; and the review decision found the rostering of the two shifts to be unreasonable management action [¶ 260].

The agreement of 17 June 2020, the Appellant's submission that it "was no longer fit for purpose" after his transition, and the surrounding matters as to it are pleaded under the agreement heading above. [¶¶ 17 to 25]

The rest recorded between those shifts. The seven-hour figure is the interval between the rostered finish at 23:00 on 17 March 2024 and the rostered commencement at 06:00 on 18 March 2024. The review decision records the Appellant's response of 9 August 2024 that the break "did not include travel time" and that, with travel factored in, the break between shifts would have been less than five hours. [¶ 251] The Appellant's account is this. His home is on the Gold Coast and the drive to and from Logan Hospital took about an hour each way at those hours. Across the two shifts about four hours were spent driving. When the 17 March shift ended at 23:00 he had been awake and working for about sixteen hours. Of the seven hours before 06:00, two were on the road and at best four to five were available for sleep. So, across the pair of shifts, the Appellant's account is: about sixteen hours of work, on two eight-hour shifts; about four hours of travel, in four drives of about an hour each; and about four hours of sleep, of the four to five hours available. The seven hours and the response of 9 August 2024 are the admitted record; the rest of that account is the Appellant's own evidence.

The codes on the shift of 17 March 2024. The shift ended at 23:00. [¶ 284] The Emergency Code Register for that day (Tab 31) records eight codes: at 3:57 a Code Grey in the ED lounge; at 5:19 a MET call to Ward 5A bed 27; at 7:48 a Code Grey in the ED corridor near short stay; at 13:48 a MET call to Ward 3B bed 2; at 14:13 a Code Grey in ED adult acute bed 15; at 19:16 a MET call to Ward 2H bed 7; at 19:20, four minutes later, a Code Blue to the same patient, Ward 2H bed 7; and at 21:16 a MET call to Ward 2Q bed 5. The last three fell in the evening of that shift.

The codes on the shift of 18 March 2024. The shift began at 06:00, seven hours after the previous one ended. [¶ 284] The register records eleven codes that day: at 8:48 a Code Grey at the QAS triage airlock; at 9:20 a MET call to Ward 6A bed 8; at 11:31 a MET call to Ward 3DR bed 14, cancelled at 11:32; at 11:35 a Neonatal MET call to Theatre 2; at 13:09 a Neonatal MET call to Birth Suites 5; at 14:38 a MET call to Ward 3DR bed 11; at 14:45 and 14:55 Code Greys at QAS triage; and at 19:15 and 19:34 Code Greys on the ED QAS ramp. Six of those fell between 06:00 and 14:00: a Code Grey, three adult MET calls (one cancelled within a minute) and two Neonatal MET calls.

The entries are those recorded in the register at Tab 31. The propositions drawn from it (paragraphs 228 to 231 of the notice: the eight entries on 17 March including the MET call and Code Blue to Ward 2H bed 7; the six entries between 06:00 and 14:00 on 18 March including the two Neonatal MET calls; and the 19 March entry "CALLED VIA SWITCHBOARD") were not admitted, and the authenticity of the copy is in issue (Schedule, Part 1, Tab 31). The employer holds its own record of the MET calls for 17 and 18 March 2024. [¶ 268]

The ordinary call load, and the Monday. In April 2025, during the Appellant's graduated return, the console took between 269 and 444 calls on a single eight-hour shift, an average of about 360, on the monthly call statistics for that period; the highest figure, 444, was the one Monday afternoon shift in that period, 14 April 2025. On the Appellant's account the Monday early shift is the peak of the week: the weekend's backlog, the resumption of clinics and elective lists and the Monday admissions arrive at the console together. The shift of 18 March 2024 was the Monday early shift, the first shift of the working week [¶ 227], and it followed a break of seven hours. [¶ 284]

3(c) Sick leave after the consecutive shifts (19 March 2024). The Leave Takings Report produced by Metro South Health in respect of 19 March 2024 records the Leave Category as "Sick", the Leave Type as "Sick Leave", the Time Code as "SCK", the Leave Taken as 7.60 hours and the Leave Status as approved. [¶ 235]

The Respondent pleads that the Appellant took leave on 19 March 2024 but says this was paid leave, and that pursuant to clause 18.10 of the Award he is not entitled to fatigue leave because he was not performing overtime. [¶¶ 236 to 237] The Respondent's outlines served 24 September 2026 give notice that its payroll

witness will say the Award's fatigue provision "applies to breaks around overtime, not between two ordinary shifts". On that account, no fatigue-leave entitlement under clause 18.10 arose from the short break between ordinary shifts. That contention does not itself answer whether the roster or the response to fatigue was reasonable.

The codes on 19 March 2024, the day of the sick leave. Sick leave of 7.60 hours was taken that day. [¶ 235] The register records sixteen entries on 19 March 2024: at 0:17 a MET call to Ward MIU bed 21; at 1:41 a Code Grey in ED Ambi Care; at 9:27 a Paediatric MET call to Ward CIU bed 16; at 10:28 a simulation MET call (adults) to Ward 7C bed 3; at 11:30 a MET call to Ward 2I bed 20; at 11:34 a MET call to the same patient, Ward 2I bed 20, annotated "CALLED VIA SWITCHBOARD" [sic]; at 12:36 a MET call to Ward 2I bed 12; at 13:42 a MET call to Ward 3DR bed 11; at 16:28 a further MET call to Ward 3DR bed 11, cancelled at 16:29; at 16:48 a Code Grey in the ED mental health lounge; at 17:23 a further MET call to Ward 3DR bed 11; at 17:31 a Code Grey in ED Ambi Care room 2; at 18:45 a MET call to Ward 5B bed 7; at 21:28 a Code Grey in the ED mental health lounge; and at 22:25 a Code Grey at ED QAS triage. Six of those fell between 06:00 and 14:00: a Paediatric MET call, a simulation, and four adult MET calls. Ward 3DR bed 11, the subject of a MET call at 14:38 on the shift of 18 March, was the subject of further MET calls at 13:42, 16:28 and 17:23 on 19 March.

3(d) Fatigue enquiry and refusal (8 April to 1 May 2024). The review decision records: "You emailed Ms Taylor on 8 April 2024 and requested a review of your payment for the pay period between 8 March 2024 and 31 March 2024", noting the Award requirement as to a minimum ten-hour break. [¶¶ 242 to 243]

It records that she replied on 9 April 2024 and "added she also escalated your enquiry regarding fatigue leave to Human Resources to confirm policies around this", and that she "proceeded to acknowledge the importance of communication and requested that you raise your concerns with her as soon as they arise so she could action them sooner for you". [¶¶ 244 to 245]

It records that on 24 April 2024 the Appellant emailed expressing dissatisfaction that it had been more than two weeks without any response or overtime payment. [¶ 246]

The Respondent does not allege that any response was made to the fatigue enquiry between 9 April and 1 May 2024; the enquiry of 8 April was answered after 23 days. [¶¶ 249 to 250]

It records that she responded on 1 May 2024 stating that after a consultation with payroll and Human Resources the request for fatigue payment for 18 March 2024 would not be processed due to the existing 8-hour agreement, and noting that he was able to terminate that agreement going forward. [¶¶ 247 to 248]

It records that payroll was, as at 1 May 2024, still reviewing his entitlements; that the employer did not mention his shift on 18 March 2024; and that there was uncertainty between him and the employer regarding whether the 8-hour agreement continued to apply. [¶¶ 253 to 255]

So the enquiry of 8 April 2024 was answered on 1 May 2024, 23 days later, with a refusal. In between were the acknowledgment of 9 April, the escalation to Human Resources, and the Appellant's email of 24 April; no other response is alleged. [¶¶ 242 to 250] So the day after the consecutive shifts was recorded against the Appellant's own sick leave [¶ 235], and the Respondent says no fatigue leave was due for it [¶¶ 236 to 237]. The refusal relied on an agreement signed at a time the Respondent does not allege the Manager held that role, and never reviewed; the Respondent does not allege that he was told, before that day, that he could end it "going forward". [¶¶ 21 to 22, 25, 242 to 250]

3(e) Roster line error and proposed alternatives (16 to 26 April 2024). On 26 April 2024 at 1:52 pm the Director wrote with the subject "Roster Concerns", referring to a meeting on 16 April 2024 at which the Appellant had raised a roster line ending with night shifts followed by three days off and then returning to night shifts, and stating that the Manager "was working to fix this error and would get in touch with you about what alternative shifts she could offer". [¶¶ 211 to 212]

The Respondent does not allege that she got in touch with him about alternative shifts following that meeting. [¶ 213]

3(f) Fatigue toolkit raised and HR advice awaited (1 to 8 May 2024). On 1 May 2024 at 1:18 pm the Appellant referred to section 4, clause 10.4.1 of the Operations Manual, which "includes a toolkit required by management to manage fatigue and implement appropriate protocols and adhere to the workplace health and safety act", and stated: "I forwarded the toolkit to Chloe last week for review and action but have yet to receive feedback". [¶¶ 214 to 215]

The Respondent does not allege that the toolkit was reviewed or that any feedback was provided before 18 June 2024. [¶ 216]

On 8 May 2024 at 9:08 am the Director replied that she was following up with HR for further advice and would get back to him as soon as possible. [¶ 217]

3(g) Risk rating and roster decision recorded (10 May 2024). On 10 May 2024 at 2:08 pm the Director wrote to Human Resources forwarding his email, stating: "I think he is trying to raise that he has concerns over how his manager is rostering for the Switchboard team and how it is impacting on staff fatigue, or more specifically his fatigue." [¶ 218]

She stated that she and the Manager had run through the Roster Risk Assessment Matrix and "we say at best there would be a rating of 11 which is moderate". [¶ 219]

She stated: "I acknowledge there has been a few rostering errors made by Chloe with regards to Cory's line in past rosters". [¶ 220]

She stated: "Cory's roster will not be considered, however Cory is not yet aware of this as we cannot advise what is the roster we are planning on going with before we send out the implementation plan for this business case for change". [¶ 221] Eleven days later, on 21 May 2024, she asked him which particular changes or directives had concerned him. [¶ 80]

3(h) HR follow-up with the fatigue guideline (20 May 2024). On 20 May 2024 the Director sent Human Resources the Queensland Health Fatigue Risk Management Systems Implementation Guideline, qh-gdl-401-3.3. [¶¶ 222 to 223] Ten days earlier she had written that she and the Manager had run through the Roster Risk Assessment Matrix and "we say at best there would be a rating of 11 which is moderate". [¶ 219] The Respondent disclosed the guideline, with that email chain, on 14 July 2025 (Tab 8A). The following passages are from that disclosed copy.

Supporting records about the fatigue arrangements

Implementation and records of fatigue risk management. By letter of 5 June 2026, reference K-LM26/729, signed by the Chief Executive and addressed to the Commission, Metro South Health stated that the requested fatigue risk management documents "do not exist", that mandatory Fatigue Risk Management System training "only applies to health practitioners and clinical assistants" and that Switchboard staff are non-clinical. [¶¶ 263, 265] It stated that "The implementation of fatigue risk management assessment at Switchboard Logan Hospital occurred after 30 June 2024". [¶ 264] It stated that "there have been no 'consequential' changes to operating procedures over the period requested". [¶ 266] The Respondent does not allege that any fatigue risk assessment was conducted, that fatigue risk management training was provided, that fatigue risk management assessment was implemented at the Switchboard, or that any change was made to the operating procedures of the Switchboard as a consequence of any employee complaint, at any time before 30 June 2024. [¶¶ 269 to 272]

The guideline defines fatigue as "A state of impaired physical and/or mental performance and lowered alertness arising as a result or combination of physical and mental work, health and psychosocial factors or inadequate restorative sleep", a fatigue risk management system as "An integrated set of management practices and procedures for monitoring and managing the risks posed to health, safety and wellbeing by fatigue", and "Defences in depth" as a "Hazard identification and risk control model that applies a series of layered mechanisms to minimise the occurrence of fatigue related incidents". [Guideline, glossary, p 29]

It provides that "Level 4 controls relate to error tolerance and containment" and that "Error tolerance relates to built-in redundancies that decrease the likelihood that a fatigued individual will make an error which may have catastrophic consequences." [p 21] It lists among its related documents the Hospital and Health Services General Employees (Queensland Health) Award - State 2015 and the "I1 (QH-POL-171) Fatigue risk management policy". [p 27, section 9]

On consultation and training, the guideline states: "there is a legislative obligation to consult with workers in relation to hazard identification and the making of decisions about how associated risks can be managed. Consultation requirements for Queensland Health are described in Health, safety and wellbeing consultation standard (QH-IMP-401-2:2020)." It states that information, training and instruction "must have regard for: the nature of the work carried out; the nature of the risks associated with the work; the control measures implemented", and that "where a work unit or an occupational stream has been identified at risk of fatigue, the training provided must consider the above criteria". Its example training matrix (Figure 11) lists, as those

to be trained, "Staff at risk of fatigue", "Rostering staff" and "Manager, supervisors, executives", with learning outcomes including "Rostering". [p 26, section 7.1]

Its supporting documents include the "Fatigue risk assessment tool", the "Individual fatigue likelihood calculator" and the "Samn-Perelli fatigue checklist". [pp 27 to 28, section 9] It defines a "Worker" by reference to section 7 of the WHS Act, including "An employee", and states that "The Department of Health and each of the HHSs are considered to be PCBU's". [pp 29 to 30, section 10] Its version control records that version 1.0 of 10 November 2021 replaced the "Fatigue Risk Management System Resource Pack December 2018 v. 3". [p 30, section 11] Its references include "Queensland Ombudsman (2006). The Neville Report". [p 31, section 12]

So the guideline treated the Award under which the Appellant was employed as a supporting document. It treated every employee of a Hospital and Health Service as a worker to whom it applied. It called for consultation with workers on fatigue hazards, and for training of rostering staff and managers. A resource pack had preceded it since December 2018. The Chief Executive's letter states that fatigue risk management assessment at the Switchboard was implemented after 30 June 2024 and that the mandatory training "only applies to health practitioners and clinical assistants". [¶¶ 264 to 265] The Respondent does not allege that any fatigue risk management training was provided to the Appellant, or that any fatigue risk assessment of his position was conducted, before 30 June 2024. [¶¶ 269 to 270]

The guideline the Director sent to Human Resources on 20 May 2024 is the guideline the Chief Executive's letter of 5 June 2026 says was implemented at the Switchboard only "after 30 June 2024" [¶ 264], and the training for which "only applies to health practitioners and clinical assistants" [¶ 265]. The fatigue risk of the Appellant's roster had been assessed by the Director and the Manager as moderate on 10 May 2024 [¶ 219]; no fatigue risk assessment of his position is alleged to have been conducted before 30 June 2024 [¶ 269].

Where the framework came from. The guideline's references list "Queensland Ombudsman (2006). The Neville Report". [p 31, section 12] Queensland Health's Policy on Medical Fatigue Risk Management, which took effect on 1 July 2008, followed that report (Queensland Ombudsman, The Neville Report Update, 2011), and the resource pack of December 2018 and the 2021 guideline that replaced it apply the system to every "worker" of a Hospital and Health Service, including "An employee" [pp 29 to 30, section 10; p 30, section 11]; the Switchboard is where the codes are received and paged. [¶ 8]

Findings recorded in the decision under appeal

The findings in the decision under appeal. The review decision states: "The break between the shift on 17 March 2024 and 18 March 2024 equated to 7 hours." [¶ 258]

It states that even if the Appellant was allowed to leave early on 17 March 2024 as suggested by the employer, he left a maximum of thirty minutes early, which meant he still did not receive a minimum 8-hour break. [¶ 259]

It states: "Based on this, I find the rostering of these two shifts amounted to unreasonable management action given that it was in direct contradiction to the award and the 8-hour agreement." [¶ 260]

It states, under the heading "Conclusion", that the Appellant "sustained a personal injury of a psychological nature". [¶ 261]

It states that his injury "arose out of employment, to the extent that it arose out of factors 2, 3 and 4, where employment was a significant contributing factor". [¶ 262]

The Respondent's pleaded answer and subsequent roster proposal

The Respondent pleads that the shift was separated by only a 7-hour break, that this was a result of human error and not intentional or repeated, that in June 2020 the Appellant signed an agreement allowing an 8-hour break, and that "the appellant could refuse shifts at anytime". [¶¶ 226, 232 to 233] The Respondent's outlines of evidence served 24 September 2026 give notice that its witnesses will say the break "was the result of human error", and that the advice returned within payroll was that "what had occurred was better characterised as a rostering practice issue for the line manager rather than an unpaid entitlement".

The Respondent's pleaded position on the roster. On 18 February 2026 the Respondent denied paragraphs of the Appellant's earlier notice on the stated grounds "because the comparator used does not represent a true comparator in the circumstances and because the roster was equitable". [¶ 299]

The Consultation Paper of November 2024 states that the proposed roster "is intended to introduce a more equitable roster for Switchboard Services, Logan Hospital staff". [¶ 171]

It states that the proposed roster includes "Redistribution of nights for greater equity based on FTE" and a "Rotational roster for predictability and equity to all staff ensuring fair distribution of penalties". [¶¶ 172 to 173]

It states that it includes an additional eight-hour Saturday shift to assist with call volume and the extension of a five-hour weekday shift to eight hours. [¶¶ 174 to 175]

The Respondent's pleaded answer to this stressor. Beyond the agreement and the sick-leave matters at 3(c), the Respondent does not allege that the Appellant was subject to any disciplinary process, or that his work performance was the subject of any formal performance management process, before 18 June 2024, nor does it describe any communication with him before that date as a warning. [¶¶ 300 to 302] Paragraph 27 of its amended statement of facts and contentions does not identify, by particular, date, document or cross-reference, the management action relied upon. [¶¶ 238, 303] The employer's own account of its complaint handling is that complaints "are made directly to the Line Manager of Switch Board and managed solely via email or verbally with the complainant", and no change to operating procedures as a consequence of any employee complaint is alleged. [¶¶ 267, 272]

The medical link - Stressor 3. Tab M1 records the 1 July 2024 report that the roster was being manipulated so the Appellant worked "lates then earlies". Tab M4 later records workplace stress involving management and rostering. Tab M8 is the review decision recording the seven-hour break and the finding that the rostering of the 17 and 18 March shifts was unreasonable management action. Tab M7 is relied on only for continuing effect and capacity in 2026. The medical material is not relied on to prove the roster events. Those events are proved by the admitted facts above about the roster, the policy, the correspondence and the review decision.

The medical case as a whole. The medical material does not give a separate diagnosis for each pleaded event. The Appellant relies on one psychological injury arising from the whole course of conduct. Repeating an admitted fact under more than one stressor therefore serves a purpose and is not double counting. It allows each stressor to be decided on its own, while keeping the whole course of conduct that was described to the treating doctors.

3. What happened after the injury, and the record made at the time

3.1 After the injury - subsequent conduct, relied on for context and continuing effect, not as cause.

The work capacity certificates certify no functional capacity from 1 July to 6 October 2024. Matters after the date of injury are not relied on as causes of it; those stated in this paragraph are stated for context and continuing effect only, as they were at Part B.3 of the Amended Statement filed 7 April 2026. (a) On 12 and 16 July 2024, while on certified leave, the Appellant was required to attend meetings convened by management. (b) On 8 October 2024 the employer terminated his employment on the basis of abandonment of employment, following certificates of no functional capacity from 1 July to 6 October 2024; he applied for reinstatement (TD/2024/110) and was reinstated. (c) The Respondent obtained the Appellant's medical records under the Form 29 signed 4 July 2025 without serving that notice on him; on 18 February 2026 it admitted that it did not serve it (Form 24, paragraph 25). Those are the records relied on at Part B.1. The Respondent's amended statement of 13 May 2026 says of (a) to (c) that they post-date the injury and are not relevant; the Appellant relies on them as stated above, and on (c) for the provenance of the medical record. (d) The Employee Capability Checklist completed by Dr Day Hong Ma on 3 July 2026 records current capacity and restrictions and the continuing effect of the injury, including "symptom exacerbation on exposure to the identified workplace stressors" [Tab M7]; it is relied on for the effect of the injury and for capacity only, not for cause. (e) The Appellant's present position is this. On 2 July 2026 the employer advised that he could not return to work until a completed Employee Capability Checklist was provided; Dr Ma completed it on 3 July 2026, certifying him fit with restrictions, and the employer has directed him not to attend work since that date, pending its request of 31 July 2026 for further medical information. He has received no wages since 13 July 2026. The treating psychiatrist wrote to the employer on 13 August 2026 and, on 19 August 2026, asked the basis of the request. On 24 August 2026 the Appellant authorised the psychiatrist to communicate with the employer on his capacity for work. As at 5 September 2026 the enquiry of 19 August 2026 had not been answered. These are the Appellant's own evidence and are not relied on as causes of the injury.

3.2 What the Appellant provided to WorkCover Queensland, and when. The Respondent's amended List of Documents of 14 August 2026 records that the Appellant provided to WorkCover Queensland, at item 12, an email of 12 July 2024 with an attachment described as "Event overview - undated"; at item 14, an email of

18 July 2024 with an attachment described as "Witness statement - Carolyn Jeffrey"; at item 25, an email of 29 August 2024 described as regarding the after hours on call change, with an attachment described as "Email: After hours on call process"; at item 26, an email of 30 August 2024 described as regarding failure to consult, with an attachment described as "Email: Task change switchboard - 19/04/2024"; and at item 27, an email of 30 August 2024 described as regarding failure to consult, with an attachment described as "Email: MASPER process - 09/05/2024". It further records, at item 16, an email from Ms Carolyn Jeffrey to WorkCover Queensland of 1 August 2024 described as a follow up statement. [¶¶ 274 to 280] The documents at items 25, 26 and 27 are the three directives pleaded at Stressors 1(f), 1(g) and 1(h) above. They were provided by the Appellant to WorkCover Queensland in August 2024, before any decision on his claim and more than two years before this pleading.

3.3 The particulars bundle of 11 August 2026. The Respondent's amended statement of facts and contentions states at paragraph 11 that it does not admit the allegations in Stressor 1(a) of the Appellant's Amended Statement of 7 April 2026 "because there are no particulars or details to respond to". On 11 August 2026 the Appellant served a bundle titled "Stressor 1(a) - Particulars support bundle", comprising 30 pages and six tabs, each stating a particular of that stressor and enclosing the documents recording it. [¶¶ 281 to 282]

4. The intervals

The intervals below are worked out from admitted dates and times or from the specifically identified Payroll disclosure. They are arithmetic only. They do not depend on whether any statement in a document is true. Each is pleaded in its place in Part B.2; this table collects them.

Interval	Length	Where pleaded	Facts
Rostered finish on 17 March 2024 at 23:00 to rostered start on 18 March 2024 at 06:00. The Appellant relies on the ten-hour minimum or, alternatively, eight hours if the agreement applied; the travel account is separately identified.	7 hours	3(b); Stressor 3 minimum-break and review-decision paragraphs	¶¶ 284 to 285, 225, 234, 251, 260
MASPER Registrar's first report, 3 May 2024 at 3:06 pm, to the first reply, 9 May 2024 at 9:20 am (her second report was sent on the fifth calendar day; the reply on the sixth)	5 days, 18 hours, 14 minutes	1(h)	¶¶ 62 to 64
Integrated Respiratory Service's first email, 15 May 2024 at 11:47 am, to its second, 20 May 2024 at 11:03 am	the fifth calendar day	1(k)	¶ 92
That first email to the Appellant's escalation, 20 May 2024 at 2:05 pm	5 days, 2 hours, 18 minutes	1(k)	¶¶ 97 to 98
The Service's second email to that escalation	3 hours, 2 minutes	1(k)	¶ 97
The escalation to the reply at 4:30 pm, two hours after the office hours stated to all Switchboard staff on 17 May 2024 had ended	2 hours, 25 minutes	1(k)	¶¶ 102 to 103
Fatigue enquiry of 8 April 2024 to the substantive response of 1 May 2024; acknowledged on 9 April, with no further response alleged before 1 May	23 days	3(d)	¶¶ 249 to 250
Third submission of the special pandemic leave request, 29 February 2024 at 11:07:28, to its approval at 11:21:03	13 minutes, 35 seconds	1(e), 2(b)	¶ 124
Creation of the draft, 20 February 2024, to final approval, 1 March 2024	10 days, 4 hours, 40 minutes, 20 seconds	1(e), 2(b)	¶ 125
Removal of database access, 18 July 2023, to 18 June 2024; no restoration alleged	11 months	1(b)	¶ 55
23 August 2023 to 17 May 2024; no fixed office hours alleged to have been stated	Approximately 9 months	1(d), 1(l)	¶ 73
Payroll's direction, 3 May 2024, to the pay date of the payslip carrying three of the four corrections, 5 June 2024	33 days	2(n)	Payroll disclosure, July 2025
That direction to the pay date of the payslip carrying the fourth, 2 July 2025 (amendment dated 30 June 2025)	425 days	2(o)	Payroll disclosure, July

Interval	Length	Where pleaded	Facts
			2025
The Appellant's last shift, 3 June 2024, to the first of those pay dates	2 days	2(n); Part B.1.4	-
The pay enquiry of 8 April 2024 to the reply of 9 April	1 day	2(g), 3(d)	¶¶ 242, 244
That reply to the Appellant's follow-up of 24 April 2024	15 days	2(g), 3(d)	¶¶ 244, 246
That follow-up to the refusal of 1 May 2024	7 days	2(g), 3(d)	¶¶ 246, 247
The refusal of 1 May 2024 to Payroll's direction of 3 May 2024	2 days	2(g), 2(h)	¶¶ 247, 185
Payroll's direction to the Appellant's email to Payroll, 10 May 2024	7 days	2(h), 2(i)	¶¶ 185, 191
Payroll's direction to Payroll's email that nothing had been corrected, 13 May 2024	10 days	2(h), 2(i)	¶¶ 185, 193
Payroll's direction to the Manager's "still waiting" email, 21 May 2024	18 days	2(h), 2(l)	¶¶ 185, 195
Payroll's direction to the Manager's AVAC of 28 May 2024	25 days	2(h), 2(m)	¶¶ 185, 197, 209
That AVAC to its processing as "Part Completed", 30 May 2024	2 days	2(m)	¶ 203
The shifts of 28 February, 30 March and 1 April 2024 to the payslip that adjusted them, pay date 5 June 2024	98 days; 67 days; 65 days	2(c), 2(e), 2(f), 2(n)	¶¶ 187 to 189; Payroll disclosure, July 2025
The shift of 9 February 2024 to the payslip carrying its correction, pay date 2 July 2025	509 days	2(a), 2(o)	¶¶ 185 to 186; Payroll disclosure, July 2025
The Appellant's request for the Manager's office hours, 15 May 2024 at 1:15 pm, to the Director's request to retract it, 6:23 pm the same day	5 hours, 8 minutes	1(l)	¶¶ 74, 76
That request to the hours being stated to the department, 17 May 2024 at 9:30 am	1 day, 20 hours, 15 minutes	1(l)	¶¶ 74, 81
That request to the Director's question about which changes had concerned him, 21 May 2024 at 2:53 pm	6 days, 1 hour, 38 minutes	1(l)	¶¶ 74, 79

Particulars: paragraphs 55, 62 to 64, 73 to 74, 76, 79, 81, 92, 97 to 98, 102 to 103, 124 to 125, 185 to 197, 203, 209, 225, 234, 242 to 247, 249 to 251, 260, 284 to 285 of the notice to admit facts served 28 August 2026, admitted 8 September 2026; the Respondent's payroll disclosure of July 2025 (Schedule, Part 2).

5. Matters the Respondent does not allege, and documents it does not list

The Respondent does not allege any of the matters listed under the headings below. It has admitted this as to the state of its amended statement of facts and contentions of 13 May 2026 and its amended List of Documents of 14 August 2026. These are admissions about what those documents do and do not contain. They are not admissions that the contents of any document are true, and the qualification in the Respondent's letter of 8 September 2026 does not apply to them.

5.1 As to the directory and the changes to process. That access to the database was restored to the Appellant before 18 June 2024; that any document records consultation with Switchboard operators before the change communicated on 15 April 2024; that agreement under clause 6.2 of the Award was obtained, or a ballot conducted, before that change.

5.2 As to what followed the reports at the console. That any communication was sent to Logan Switch or the Switchboard staff about the occasions reported on 3 and 8 May 2024 before 10:15 am on 9 May 2024; that any response was made to the Integrated Respiratory Service before 20 May 2024, or before 2:05 pm on that day; that any document created on 20 May 2024 records the discussion referred to in the reply of that

afternoon; that any communication was sent by either named person to that Service on that day; that the contact-details document was amended on or before 20 May 2024; that the Switchboard staff were notified of the modifications made to it on 22 February 2024; or that the Appellant was rostered to work after 2:05 pm on 20 May 2024.

5.3 As to the office hours. That fixed office hours were stated to the Switchboard staff at any time between 23 August 2023 and 17 May 2024.

5.4 As to the Communication Book. That the entry removed, or its author, is identified; that the removed page has been located or that any copy of it exists; that any entry made by the Appellant remains in the book; or that the book ceased to be in the possession of Metro South Health during the Appellant's employment. No proposition about whether the book is listed in the amended List of Documents is relied upon in this paragraph; fact 154 was not admitted.

5.5 As to the special pandemic leave. That any other Switchboard employee was required to submit such a request personally through myHR in February 2024.

5.6 As to the roster and fatigue. That the Switchboard Manager got in touch with the Appellant about alternative shifts after the meeting of 16 April 2024; that the fatigue toolkit was reviewed or feedback given before 18 June 2024; that the consecutive shifts of 17 and 18 March 2024 arose from a staff-initiated shift swap; that any response was made to the fatigue enquiry between 9 April and 1 May 2024; that any fatigue risk assessment was conducted, that any fatigue risk management training was provided, or that fatigue risk management assessment was implemented at the Switchboard, before 30 June 2024; or that any change was made to the operating procedures of the Switchboard as a consequence of any employee complaint over the period 1 December 2023 to 30 June 2024.

5.7 As to the Appellant's performance and conduct. That he was subject to any disciplinary process, or that his work performance was the subject of any formal performance management process, before 18 June 2024; nor does the Respondent describe any communication with him before that date as a warning. Paragraph 27 of its statement of facts and contentions does not identify, by particular, date, document or cross-reference, the management action relied upon for the contention in that paragraph.

Particulars: paragraphs 16, 21 to 22, 25, 55, 69, 73, 90, 93, 99, 105 to 110, 142, 150 to 153, 166, 180 to 181, 213, 216, 234, 250, 269 to 273, 300 to 303 of the notice to admit facts served 28 August 2026, admitted 8 September 2026. Paragraph 154 of that notice is not admitted.

PART C - CONTENTIONS

1. The onus of proof, accepted. The Appellant accepts that it is for him to prove, on the balance of probabilities, that he sustained an injury within section 32, and that the exclusion in section 32(5)(a) does not apply to the injury proved. Whether management action was reasonable is judged objectively. *Prizeman v Q-COMP* [2005] QIC 53; (2005) 180 QGIG 481 states that the focus is on what the employer actually did, not on how the worker perceived it. The contrary contention in the Amended Form 9A filed 7 April 2026 is withdrawn.

2. The admitted record is the factual basis for that assessment. The Appellant does not ask the Commission to find conduct unreasonable from labels alone. The Respondent has admitted, to the extent recorded in its response of 8 September 2026, the facts drawn from the documents set out in Part B. They include: that database access was removed and is not alleged to have been restored before 18 June 2024; that no document records consultation with Switchboard operators before the 15 April 2024 change; that the first report of calls reaching the wrong medical team came nearly six days before the first identified communication to Switchboard staff about them; that the record of 20 May 2024 contains the gaps identified at Stressor 1(k); and that the employer's letter of 5 June 2026 states that the identified fatigue records do not exist, that the mandatory training applied only to health practitioners and clinical assistants, that fatigue assessment at the Switchboard began after 30 June 2024, and that there were no consequential changes to operating procedures over the period requested.

3. Injury under section 32(1). The Appellant contends that he sustained a personal injury of a psychological nature arising out of, or in the course of, his employment, with employment a significant contributing factor. The injury is the diagnosed condition identified in Part A. It is pleaded as arising from the whole course of conduct in Part B.2, not from an event on a single day. The Respondent's review decision recorded both a psychological injury and employment as a significant contributing factor, while rejecting the claim under section 32(5). Section 32(1) requires that employment be a significant contributing factor, not the only one. The Employee Capability Checklist of 3 July 2026 (Part B.3.1) records symptom exacerbation on exposure to the identified workplace stressors. It is relied on for the continuing effect of the injury only, not as a cause. No aggravation is claimed.

4. Section 32(5)(a) - working conditions that are not management action. The Appellant contends that some of the circumstances in Stressor 1 are conditions in which the work had to be done, not management action. They are: the position stayed responsible for accurate contact and paging information, a function admitted to be critical to patient safety, while operators could not directly amend the database (1(b)); calls, including a MET-call enquiry, were sent to the wrong medical team (1(h)); a clinical service reported that it could not help patients or staff because of a wrong directory entry (1(k)); and operational complaints were, on the employer's own account, handled in the way recorded in its letter of 5 June 2026. The Commission is asked first to decide which circumstances are management action and which are not, and then to apply the exclusion. To the extent those circumstances are not management action and were a significant contributing factor, the injury did not arise out of management action, and section 32(5)(a) does not exclude it (paragraphs 6 and 7 below).

5. Section 32(5)(a) - if management action, whether it was reasonable and taken in a reasonable way. To the extent the pleaded circumstances are management action, the Appellant relies on the admitted sequence, timing and gaps in the documents in Part B. In particular: the 15 April process change was effective on the day it was notified, and no document of prior consultation is listed; pay corrections for the identified fortnights were sent back through the line-manager process, one later claim is still recorded as "Part Completed", and two identified February AVAC references do not appear in the report; the paid Special Pandemic Leave entitlement in Stressor 2(b) took three submissions and was declined twice, and the Respondent later pleaded that the supporting attachments were in fact present when the later decline was made; the fatigue enquiry was escalated to Human Resources and answered after 23 days, with an acknowledgment on 9 April and no further response alleged before 1 May; and the roster decision described in the email of 10 May 2024 was not to be disclosed to the Appellant until the implementation plan was issued. The Appellant relies on the intervals in Part B.4, and on the matters the Respondent does not allege and the documents it does not list in Part B.5.

5A. The Respondent's notified evidence is consistent with the admitted record. On 24 September 2026 the Respondent served outlines of the evidence of Ms Taylor, Ms Reese, Ms Wright and Ms Earl. An outline is not evidence; the outlines are relied on only as the Respondent's own description of the matters they

concern, and the Appellant will ask each witness to confirm them. On the Respondent's own outlines: the seven-hour break was "the result of human error" and, on the advice returned within payroll, "a rostering practice issue for the line manager"; the agreement of 17 June 2020 reduced the break "from ten hours to eight", so that a seven-hour break fell below both; no fatigue-leave entitlement under clause 18.10 attached to a short break between two ordinary shifts; the second decline of the paid Special Pandemic Leave request "appears to have been made in error"; and pay corrections were the line manager's to submit. The Appellant contends that the rostering of 17 to 18 March 2024 was, on the Respondent's own description, an act of the line manager in rostering; that it is management action; and that it was not reasonable management action taken in a reasonable way, for the reasons in Stressor 3. No outline addresses the removal of database access or its restoration; the Contact & Number Changes book; any consultation preceding the change notified on 15 April 2024; the reports of the MASPER Registrar of 3 and 8 May 2024; the emails of the Integrated Respiratory Service of 15 and 20 May 2024; the amendment of the clinic contact-details document; the fatigue enquiry of 8 April 2024 or the interval to 1 May 2024; the email to Human Resources of 10 May 2024; or the absence of fatigue risk management at the Switchboard before 30 June 2024. Those matters are relied on to the extent established by the admissions; silence in an outline is not a further admission. The Respondent's list of witnesses, sent to the Registry for filing and to the Appellant on 24 September 2026, names those four witnesses, described as "lay" witnesses, and no other, and states that the Respondent "reserves its right to amend this list depending on the case presented by the appellant at the hearing". Neither the list nor the outlines names a medical or expert witness, and no expert report has been served as at 30 September 2026.

5B. Section 32(5)(b) - the injury did not arise from the Appellant's expectation or perception of management action. The circumstances relied on in Part B.2 are events and documents admitted by the Respondent, and the intervals between them are arithmetic on admitted dates and times. They are not the Appellant's expectation or perception of reasonable management action being taken against him. The Appellant contends that section 32(5)(b) has no application to the injury proved on that record.

6. Assessment as a whole, where appropriate. *Delaney v Q-COMP* [2005] QIC 11; (2005) 178 QGIG 197 supports assessing management action as a whole where its parts are closely enough connected by subject matter, time and the people involved. The Appellant relies on that approach only where the Commission considers the connection close enough. Each stressor is also pleaded on its own, so that the reasonableness question can be answered without bringing in a fact from another stressor.

7. The ultimate statutory question concerns the injury as a whole. *Workers' Compensation Regulator v Langerak* [2020] ICQ 2, as applied in *Carr v Workers' Compensation Regulator* [2022] QIRC 59 at [60], states that the ultimate question under section 32(5)(a) is whether the injury, not any one stressor, arose out of or in the course of reasonable management action taken in a reasonable way. That requires the reasonable management action to be weighed against the other employment factors, in light of the whole of the evidence. *Simon Blackwood (Workers' Compensation Regulator) v Mahaffey* [2016] ICQ 10 is consistent with the proposition that, where other employment factors also caused the disorder, the presence of reasonable management action among the causes does not by itself answer the statutory question. The review decision's finding that the rostering of 17 and 18 March 2024 was unreasonable management action is relied on as an admitted document, not as a binding decision.

8. The effect of the admissions. The admissions are made for this proceeding under rule 49. They are relied on as evidence that the documents admitted exist and of what they say, and as evidence of what the Respondent's statement of facts and contentions and List of Documents do not contain. The Appellant does not contend that any admission decides whether any conduct was reasonable; that is for the Commission.

9. The hearing de novo. The Respondent admits the contents of Review Decision 69983 and says no finding in it binds the Commission because the hearing is de novo (a fresh hearing). [¶ 295] That is accepted, and it applies to the whole of the decision. The conclusion under appeal is a conclusion of that decision, reached on the findings recorded in it.

10. The Respondent's pleaded case. Paragraph 27 of the amended statement of facts and contentions does not identify, by particular, date, document or cross-reference, the management action relied upon. No disciplinary process, formal performance management or communication described as a warning before 18 June 2024 is alleged. This admission concerns paragraph 27. It does not dispose of the reasonableness contentions pleaded elsewhere in the Respondent's statement. Those explanations are to be assessed against the dated particulars identified here.

Particulars: paragraphs 238, 295, 300 to 303 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

11. The particulars on which the Respondent's notified evidence gives no account. Read with paragraph 5A, the Respondent's outlines of 24 September 2026 give no account of the dated events at Stressor 1(b), 1(f), 1(h) and 1(k), or at Stressor 3(e) to 3(h). As to Stressor 3(b), they describe the seven-hour break as "the result of human error" and, on the advice returned within payroll, "a rostering practice issue for the line manager". As to Stressor 2(b), they describe the second decline as one that "appears to have been made in error". On those particulars the facts are admitted, and the questions are whether each was management action, whether any management action was reasonable and taken in a reasonable way, and whether it contributed to the injury.

PART D - THE QUESTIONS FOR DETERMINATION

1. Did the Appellant sustain a personal injury of a psychological nature?
2. Did that injury arise out of, or in the course of, his employment, and was his employment a significant contributing factor to it?
3. Which of the circumstances pleaded in Part B.2 are management action taken by or on behalf of the employer in connection with the Appellant's employment, and which are conditions in which the work had to be done?
4. In respect of those circumstances that are management action, was that action reasonable and was it taken in a reasonable way?
5. Does section 32(5)(a) of the Act exclude the Appellant's psychological disorder from the definition of injury, having regard to the circumstances found not to be management action, and to any management action found not to be reasonable or not to have been taken in a reasonable way?
6. Does section 32(5)(b) of the Act exclude the injury, that is, did it arise out of, or in the course of, the Appellant's expectation or perception of reasonable management action being taken against him, rather than out of the admitted circumstances themselves?

PART E - ORDERS SOUGHT

1. That the appeal be allowed.
2. That the decision of the Respondent dated 24 October 2024, Review Decision 69983, be set aside.
3. That it be declared that the Appellant sustained an injury within the meaning of section 32 of the Workers' Compensation and Rehabilitation Act 2003, being a psychological injury arising out of, or in the course of, his employment with Metro South Hospital and Health Service, employment being a significant contributing factor to that injury.
4. That the Appellant's application for compensation be accepted, and that the matter be remitted to the Respondent for assessment of the Appellant's entitlement to compensation consequent upon that acceptance.
5. Such further or other order as the Commission considers appropriate.
6. That the Respondent pay the Appellant's costs of the appeal under section 558(3) of the Act and section 191 of the Workers' Compensation and Rehabilitation Regulation 2025, including an award under section 191(3) of up to 1.5 times the amounts under section 191(2)(a), to be assessed failing agreement; alternatively, that costs be reserved.

PART F - COSTS

1. Costs of the hearing are in the Commission's discretion: section 558(3) of the Act; section 191(1) of the Workers' Compensation and Rehabilitation Regulation 2025. Costs ordinarily follow the event.
2. If the appeal is allowed, the Appellant will seek an order that the Respondent pay his costs of the appeal, assessed under section 191(2): (a) counsel's or solicitor's fees, for any period in which the Appellant is legally represented, at the costs stated in the Uniform Civil Procedure Rules 1999, schedule 2, part 2, scale C; (b) costs in relation to witnesses at the allowances stated in the Uniform Civil Procedure (Fees)

Regulation 2019, part 3, including the fees and expenses of the medical practitioners called under paragraph B.1.9; and (c) any costs in relation to bailiffs.

3. The Appellant will contend that the amounts under section 191(2)(a) are inadequate and will seek, for any period in which he is legally represented, up to 1.5 times those amounts under section 191(3), having regard to the work involved (a record of 303 facts, 298 of them admitted; the documents at Annexure A to the notice to admit; non-party disclosure under rule 64G; two notices to admit facts and a notice to admit documents; and the four outlines served by the Respondent) and to the importance, difficulty and complexity of an appeal about a psychological injury under section 32(5)(a), which requires each pleaded circumstance to be classified, each management action assessed for reasonableness, and the whole weighed.

4. On the exercise of the discretion under section 558(3), the Appellant will also rely on the course of the proceeding, including that paragraph 27 of the Respondent's amended statement of facts and contentions identifies no management action [¶ 303], that the Respondent names no medical or expert witness and has served no expert report (Part C, paragraph 5A), and that the Respondent's own review decision found the rostering of 17 and 18 March 2024 to be unreasonable management action [¶ 260].

5. If the fees and expenses payable to a medical practitioner, or to any witness who gives evidence of a professional nature, are more than the costs awarded for that witness, the Appellant will apply to the Respondent under section 192 of the Regulation for payment of an additional amount.

6. The Appellant reserves the right to rely, on the question of costs, on any offer made without prejudice save as to costs.

Dated: 1 October 2026

A handwritten signature in black ink, consisting of a large, stylized 'C' followed by a series of loops and a long horizontal stroke extending to the right.

Cory Lea Shepherd
Appellant, self-represented